A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Amanda Richards
Pinnacle Surgery

Dear Dr Richards,

Re: Mr James Cole
DOB: 16/11/1986 Phone: 0406 255 038
Address: 18 Angus Street, CLUNES, VIC, 3370
Email: jimmycole1986@gmail.com

Summary

- **Gastroscopy and biopsies of the oesophagus, stomach, and small intestine were normal.**
- **Eosinophilic oesophagitis, coeliac disease, and significant gastric pathology excluded.**
- **Mild Improvement with lifestyle modifications and current pharmacotherapy.**
- **Oesophageal manometry and neuromodulator therapy deferred for now.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with James following his recent gastroscopy. You will be pleased to know that the procedure was entirely normal. Specifically, we performed tiered biopsies which excluded eosinophilic oesophagitis, coeliac disease, and any significant inflammation or pathology in the stomach.

James reports a slow but steady improvement in his symptoms. His cough and frequent throat clearing have lessened significantly compared to our initial consultation. Furthermore, he is sleeping much better and reporting higher energy levels during the day. He attributes this improvement to the lifestyle changes we implemented, including eating dinner earlier and walking after meals, alongside his current medication regimen of pantoprazole before dinner and nizatidine at night.

We spent some time discussing his ongoing sensitivity to smoke and cooking fumes, which seems to trigger chest discomfort and an immediate sensation of flatulence. I explained to James that this is likely a manifestation of visceral hypersensitivity. Based on current literature, these symptoms are often related to an increased sensory perception of normal intestinal gas volumes rather than an objective increase in gas production.

Impression and Plan James is moving in the right direction. While we had previously discussed proceeding to oesophageal manometry if his symptoms of dysphagia and chest pressure persisted, we have agreed to defer this invasive investigation given his current clinical improvement.

We also discussed the potential role of a neuromodulator, such as amitriptyline, to address the underlying neural sensitivity in the oesophagus and throat. However, James is understandably hesitant to introduce new medications while he is feeling better. I have encouraged him to continue with his current lifestyle measures and the nizatidine, which may further assist with his throat sensitivity.

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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We will continue with a conservative approach for now. I have advised James that should his symptoms plateau or worsen, we can revisit the option of manometry or neuromodulator therapy. I will review him on an as-needed basis.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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